Completed ICSR Narrative Report

Step 1: Key clinical events extracted

Case	Patient	Suspect drug and indication	Event and onset	Action and outcome
SYN-PV-001	30 years, male	Amoxicillin 20 mg once daily for anxiety	Rash, day 17	Drug stopped. Not recovered at follow-up. D: Negative, R: Positive
SYN-PV-002	36 years, sex not provided	Atorvastatin 5 mg once daily for anxiety	Myalgia, day 1	Drug stopped. Recovered. D: Unknown, R: Negative
SYN-PV-003	34 years, female	Metformin 5 mg once daily for anxiety	Nausea, day 13	Drug stopped. Recovered. D: Negative, R: Negative
SYN-PV-004	61 years, male	Amlodipine 500 mg twice daily for anxiety	Peripheral oedema, day 21	Drug stopped. Recovered. D: Positive, R: Negative
SYN-PV-005	33 years, sex not provided	Ciprofloxacin 500 mg twice daily for infection	Tendon pain, day 15	Drug stopped. Not recovered at follow-up. D: Unknown, R: Not done
SYN-PV-006	31 years, sex not provided	Sertraline 20 mg once daily for hypertension	Dizziness, day 3	Drug stopped. Recovering. D: Positive, R: Not done
SYN-PV-007	63 years, male	Omeprazole 500 mg twice daily for hypertension	Headache, day 16	Drug stopped. Recovered. D: Negative, R: Negative
SYN-PV-008	36 years, male	Losartan 5 mg once daily for infection	Fatigue, day 2	Drug stopped. Recovering. D: Negative, R: Not done
SYN-PV-009	78 years, sex not provided	Amoxicillin 5 mg once daily for anxiety	Rash, day 7	Drug stopped. Recovering. D: Positive, R: Not done
SYN-PV-010	57 years, female	Atorvastatin 5 mg once daily for infection	Myalgia, day 21	Drug stopped. Recovered. D: Unknown, R: Negative
SYN-PV-011	23 years, sex not provided	Metformin 5 mg once daily for hypertension	Nausea, day 19	Drug stopped. Recovering. D: Unknown, R: Positive
SYN-PV-012	24 years, female	Amlodipine 5 mg once daily for hypertension	Peripheral oedema, day 4	Drug stopped. Not recovered at follow-up. D: Unknown, R: Positive

D = dechallenge, R = rechallenge. Treatment start and stop dates are not provided in any case.

Step 2: MedDRA mapping (Preferred Term and System Organ Class)

Case	Reported term	MedDRA LLT	MedDRA PT	MedDRA SOC (primary)
SYN-PV-001	Rash	Rash	Rash	Skin and subcutaneous tissue disorders
SYN-PV-002	Myalgia	Myalgia	Myalgia	Musculoskeletal and connective tissue disorders
SYN-PV-003	Nausea	Nausea	Nausea	Gastrointestinal disorders
SYN-PV-004	Peripheral oedema	Peripheral oedema	Oedema peripheral	General disorders and administration site conditions
SYN-PV-005	Tendon pain	Tendon pain	Tendon pain	Musculoskeletal and connective tissue disorders
SYN-PV-006	Dizziness	Dizziness	Dizziness	Nervous system disorders
SYN-PV-007	Headache	Headache	Headache	Nervous system disorders
SYN-PV-008	Fatigue	Fatigue	Fatigue	General disorders and administration site conditions
SYN-PV-009	Rash	Rash	Rash	Skin and subcutaneous tissue disorders
SYN-PV-010	Myalgia	Myalgia	Myalgia	Musculoskeletal and connective tissue disorders
SYN-PV-011	Nausea	Nausea	Nausea	Gastrointestinal disorders
SYN-PV-012	Peripheral oedema	Peripheral oedema	Oedema peripheral	General disorders and administration site conditions

ICSR SYN-PV-001: Amoxicillin and rash

A. Patient	
Patient	30 years, male. Initials not provided (de-identified).
Medical history	Unknown (not provided).
B. Reaction	
Reported term	Rash
MedDRA coding	PT: Rash | SOC: Skin and subcutaneous tissue disorders
Onset	17 day(s) after treatment began (calendar date unknown).
Outcome	Not recovered at follow-up
Seriousness and severity	Serious (other medically important condition), as reported. Severity not stated.
Expectedness	Known (Synthetic Core Data Sheet v1.0)
Causality (WHO-UMC)	Possible
C. Drug	
Suspect drug	Amoxicillin 20 mg once daily, oral
Indication	Anxiety (MedDRA PT: Anxiety; SOC: Psychiatric disorders)
Dates and action	Start and stop dates unknown. Drug stopped (as reported).
Dechallenge / rechallenge	Dechallenge: Negative. Rechallenge: Positive.
Concomitant medicines	None reported.
D. Reporter	
Reporter	Consumer. Name, contact and country not provided.

E. ICSR narrative

This case was reported by a consumer and concerns a 30-year-old male patient (initials not provided). The patient received Amoxicillin 20 mg once daily by the oral route for anxiety (MedDRA PT: Anxiety). The start date of treatment is not provided. Rash (MedDRA PT: Rash; SOC: Skin and subcutaneous tissue disorders) was reported 17 day(s) after treatment began. The event is reported as serious (other medically important condition); severity is not stated. The reporter stated that treatment was stopped (stop date not provided). Dechallenge was negative: the reaction did not improve after the drug was stopped. Rechallenge was recorded as positive (the reaction recurred), but the report does not describe the restart. At follow-up the patient had not recovered. No concomitant medicines were reported. Medical history is not provided and is recorded as unknown. Expectedness is known according to the Synthetic Core Data Sheet v1.0. My causality assessment (WHO-UMC) is possible. Rash is a known amoxicillin reaction and 17 days is late but possible for a delayed rash. Dechallenge is negative (rash did not go away after stopping) but rechallenge is positive, and these two do not fit together. The narrative never says the drug was restarted.

Coding and data notes. 20 mg once daily is much lower than normal amoxicillin dosing and anxiety is not an amoxicillin indication, so there may be a data entry error. Reported by a consumer, not confirmed by a doctor. Rash type and severity are unknown.

ICSR SYN-PV-002: Atorvastatin and myalgia

A. Patient	
Patient	36 years, sex not provided. Initials not provided (de-identified).
Medical history	Unknown (not provided).
B. Reaction	
Reported term	Myalgia
MedDRA coding	PT: Myalgia | SOC: Musculoskeletal and connective tissue disorders
Onset	1 day(s) after treatment began (calendar date unknown).
Outcome	Recovered
Seriousness and severity	Non-serious, as reported. Severity not stated.
Expectedness	Known (Synthetic Core Data Sheet v1.0)
Causality (WHO-UMC)	Unlikely
C. Drug	
Suspect drug	Atorvastatin 5 mg once daily, oral
Indication	Anxiety (MedDRA PT: Anxiety; SOC: Psychiatric disorders)
Dates and action	Start and stop dates unknown. Drug stopped (as reported).
Dechallenge / rechallenge	Dechallenge: Unknown. Rechallenge: Negative.
Concomitant medicines	None reported.
D. Reporter	
Reporter	Pharmacist. Name, contact and country not provided.

E. ICSR narrative

This case was reported by a pharmacist and concerns a 36-year-old patient (sex not provided) (initials not provided). The patient received Atorvastatin 5 mg once daily by the oral route for anxiety (MedDRA PT: Anxiety). The start date of treatment is not provided. Myalgia (MedDRA PT: Myalgia; SOC: Musculoskeletal and connective tissue disorders) was reported 1 day(s) after treatment began. The event is reported as non-serious; severity is not stated. The reporter stated that treatment was stopped (stop date not provided). Dechallenge is unknown. Rechallenge was recorded as negative: the reaction did not recur. At follow-up the patient was recovered. No concomitant medicines were reported. Medical history is not provided and is recorded as unknown. Expectedness is known according to the Synthetic Core Data Sheet v1.0. My causality assessment (WHO-UMC) is unlikely. Myalgia is a known statin effect, but onset after 1 day is very early and rechallenge was negative (it did not come back). The patient recovered, but when this happened compared to stopping the drug is not given, so I kept dechallenge as unknown.

Coding and data notes. 5 mg is below the normal atorvastatin dose range and anxiety is not an indication. Muscle pain from anxiety or exertion cannot be ruled out. Sex, CK value and severity are unknown.

ICSR SYN-PV-003: Metformin and nausea

A. Patient	
Patient	34 years, female. Initials not provided (de-identified).
Medical history	Unknown (not provided).
B. Reaction	
Reported term	Nausea
MedDRA coding	PT: Nausea | SOC: Gastrointestinal disorders
Onset	13 day(s) after treatment began (calendar date unknown).
Outcome	Recovered
Seriousness and severity	Serious (hospitalisation), as reported. Severity not stated.
Expectedness	Known (Synthetic Core Data Sheet v1.0)
Causality (WHO-UMC)	Unlikely
C. Drug	
Suspect drug	Metformin 5 mg once daily, oral
Indication	Anxiety (MedDRA PT: Anxiety; SOC: Psychiatric disorders)
Dates and action	Start and stop dates unknown. Drug stopped (as reported).
Dechallenge / rechallenge	Dechallenge: Negative. Rechallenge: Negative.
Concomitant medicines	Vitamin D (dose, dates and indication unknown).
D. Reporter	
Reporter	Consumer. Name, contact and country not provided.

E. ICSR narrative

This case was reported by a consumer and concerns a 34-year-old female patient (initials not provided). The patient received Metformin 5 mg once daily by the oral route for anxiety (MedDRA PT: Anxiety). The start date of treatment is not provided. Nausea (MedDRA PT: Nausea; SOC: Gastrointestinal disorders) was reported 13 day(s) after treatment began. The event is reported as serious (hospitalisation); severity is not stated. The reporter stated that treatment was stopped (stop date not provided). Dechallenge was negative: the reaction did not improve after the drug was stopped. Rechallenge was recorded as negative: the reaction did not recur. At follow-up the patient was recovered. The only concomitant medicine reported was vitamin d (dose, dates and indication unknown). Medical history is not provided and is recorded as unknown. Expectedness is known according to the Synthetic Core Data Sheet v1.0. My causality assessment (WHO-UMC) is unlikely. Nausea is a known metformin effect and 13 days is acceptable. But dechallenge and rechallenge are both negative, which goes against the drug as the cause. The patient is also recorded as recovered even though dechallenge is negative.

Coding and data notes. The case is serious (hospitalisation) but the reason for admission is not given, so I cannot say it was caused by the nausea. 5 mg dose and anxiety indication do not fit metformin. Reported by a consumer.

ICSR SYN-PV-004: Amlodipine and peripheral oedema

A. Patient	
Patient	61 years, male. Initials not provided (de-identified).
Medical history	Unknown (not provided).
B. Reaction	
Reported term	Peripheral oedema
MedDRA coding	PT: Oedema peripheral | SOC: General disorders and administration site conditions
Onset	21 day(s) after treatment began (calendar date unknown).
Outcome	Recovered
Seriousness and severity	Serious (other medically important condition), as reported. Severity not stated.
Expectedness	Known (Synthetic Core Data Sheet v1.0)
Causality (WHO-UMC)	Possible
C. Drug	
Suspect drug	Amlodipine 500 mg twice daily, oral
Indication	Anxiety (MedDRA PT: Anxiety; SOC: Psychiatric disorders)
Dates and action	Start and stop dates unknown. Drug stopped (as reported).
Dechallenge / rechallenge	Dechallenge: Positive. Rechallenge: Negative.
Concomitant medicines	None reported.
D. Reporter	
Reporter	Pharmacist. Name, contact and country not provided.

E. ICSR narrative

This case was reported by a pharmacist and concerns a 61-year-old male patient (initials not provided). The patient received Amlodipine 500 mg twice daily by the oral route for anxiety (MedDRA PT: Anxiety). The start date of treatment is not provided. Peripheral oedema (MedDRA PT: Oedema peripheral; SOC: General disorders and administration site conditions) was reported 21 day(s) after treatment began. The event is reported as serious (other medically important condition); severity is not stated. The reporter stated that treatment was stopped (stop date not provided). Dechallenge was positive: the reaction improved after the drug was stopped. Rechallenge was recorded as negative: the reaction did not recur. At follow-up the patient was recovered. No concomitant medicines were reported. Medical history is not provided and is recorded as unknown. Expectedness is known according to the Synthetic Core Data Sheet v1.0. My causality assessment (WHO-UMC) is possible. Peripheral oedema is a known amlodipine effect and depends on dose. Onset at 21 days is reasonable and dechallenge is positive, with recovery. Rechallenge is negative though, which does not fit, so I did not go to probable.

Coding and data notes. 500 mg twice daily is about 100 times the usual maximum dose, so this is either a unit or typing error or a possible overdose that needs checking. Anxiety is not an amlodipine indication.

ICSR SYN-PV-005: Ciprofloxacin and tendon pain

A. Patient	
Patient	33 years, sex not provided. Initials not provided (de-identified).
Medical history	Unknown (not provided).
B. Reaction	
Reported term	Tendon pain
MedDRA coding	PT: Tendon pain | SOC: Musculoskeletal and connective tissue disorders
Onset	15 day(s) after treatment began (calendar date unknown).
Outcome	Not recovered at follow-up
Seriousness and severity	Serious (other medically important condition), as reported. Severity not stated.
Expectedness	Known (Synthetic Core Data Sheet v1.0)
Causality (WHO-UMC)	Possible
C. Drug	
Suspect drug	Ciprofloxacin 500 mg twice daily, oral
Indication	Infection (MedDRA PT: Infection; SOC: Infections and infestations)
Dates and action	Start and stop dates unknown. Drug stopped (as reported).
Dechallenge / rechallenge	Dechallenge: Unknown. Rechallenge: Not done.
Concomitant medicines	None reported.
D. Reporter	
Reporter	Pharmacist. Name, contact and country not provided.

E. ICSR narrative

This case was reported by a pharmacist and concerns a 33-year-old patient (sex not provided) (initials not provided). The patient received Ciprofloxacin 500 mg twice daily by the oral route for infection (MedDRA PT: Infection). The start date of treatment is not provided. Tendon pain (MedDRA PT: Tendon pain; SOC: Musculoskeletal and connective tissue disorders) was reported 15 day(s) after treatment began. The event is reported as serious (other medically important condition); severity is not stated. The reporter stated that treatment was stopped (stop date not provided). Dechallenge is unknown. Rechallenge was not done. At follow-up the patient had not recovered. No concomitant medicines were reported. Medical history is not provided and is recorded as unknown. Expectedness is known according to the Synthetic Core Data Sheet v1.0. My causality assessment (WHO-UMC) is possible. Tendon pain is a known ciprofloxacin (fluoroquinolone) reaction. Onset at 15 days, infection as indication, a normal dose and no co-medicines all fit. But dechallenge is unknown and rechallenge was not done, so I could not confirm a response to stopping the drug.

Coding and data notes. Tendon problems recover slowly, so 'not recovered' does not tell much. Activity level, the infection itself and steroid use are not described. Sex unknown. It could be upgraded to probable if dechallenge is confirmed.

ICSR SYN-PV-006: Sertraline and dizziness

A. Patient	
Patient	31 years, sex not provided. Initials not provided (de-identified).
Medical history	Unknown (not provided).
B. Reaction	
Reported term	Dizziness
MedDRA coding	PT: Dizziness | SOC: Nervous system disorders
Onset	3 day(s) after treatment began (calendar date unknown).
Outcome	Recovering
Seriousness and severity	Serious (other medically important condition), as reported. Severity not stated.
Expectedness	Known (Synthetic Core Data Sheet v1.0)
Causality (WHO-UMC)	Probable
C. Drug	
Suspect drug	Sertraline 20 mg once daily, oral
Indication	Hypertension (MedDRA PT: Hypertension; SOC: Vascular disorders)
Dates and action	Start and stop dates unknown. Drug stopped (as reported).
Dechallenge / rechallenge	Dechallenge: Positive. Rechallenge: Not done.
Concomitant medicines	Paracetamol (dose, dates and indication unknown).
D. Reporter	
Reporter	Physician. Name, contact and country not provided.

E. ICSR narrative

This case was reported by a physician and concerns a 31-year-old patient (sex not provided) (initials not provided). The patient received Sertraline 20 mg once daily by the oral route for hypertension (MedDRA PT: Hypertension). The start date of treatment is not provided. Dizziness (MedDRA PT: Dizziness; SOC: Nervous system disorders) was reported 3 day(s) after treatment began. The event is reported as serious (other medically important condition); severity is not stated. The reporter stated that treatment was stopped (stop date not provided). Dechallenge was positive: the reaction improved after the drug was stopped. Rechallenge was not done. At follow-up the patient was recovering. The only concomitant medicine reported was paracetamol (dose, dates and indication unknown). Medical history is not provided and is recorded as unknown. Expectedness is known according to the Synthetic Core Data Sheet v1.0. My causality assessment (WHO-UMC) is probable. Dizziness 3 days after starting is a reasonable time and a known sertraline effect. Dechallenge is positive and the patient is recovering. The only co-medication is paracetamol and the report is from a physician.

Coding and data notes. Hypertension is not a sertraline indication, so the real indication is unclear and hypertension could also cause dizziness. 20 mg is an unusual dose. Sex unknown. If another cause is found I would lower it to possible.

ICSR SYN-PV-007: Omeprazole and headache

A. Patient	
Patient	63 years, male. Initials not provided (de-identified).
Medical history	Unknown (not provided).
B. Reaction	
Reported term	Headache
MedDRA coding	PT: Headache | SOC: Nervous system disorders
Onset	16 day(s) after treatment began (calendar date unknown).
Outcome	Recovered
Seriousness and severity	Serious (other medically important condition), as reported. Severity not stated.
Expectedness	Known (Synthetic Core Data Sheet v1.0)
Causality (WHO-UMC)	Unlikely
C. Drug	
Suspect drug	Omeprazole 500 mg twice daily, oral
Indication	Hypertension (MedDRA PT: Hypertension; SOC: Vascular disorders)
Dates and action	Start and stop dates unknown. Drug stopped (as reported).
Dechallenge / rechallenge	Dechallenge: Negative. Rechallenge: Negative.
Concomitant medicines	Paracetamol (dose, dates and indication unknown).
D. Reporter	
Reporter	Consumer. Name, contact and country not provided.

E. ICSR narrative

This case was reported by a consumer and concerns a 63-year-old male patient (initials not provided). The patient received Omeprazole 500 mg twice daily by the oral route for hypertension (MedDRA PT: Hypertension). The start date of treatment is not provided. Headache (MedDRA PT: Headache; SOC: Nervous system disorders) was reported 16 day(s) after treatment began. The event is reported as serious (other medically important condition); severity is not stated. The reporter stated that treatment was stopped (stop date not provided). Dechallenge was negative: the reaction did not improve after the drug was stopped. Rechallenge was recorded as negative: the reaction did not recur. At follow-up the patient was recovered. The only concomitant medicine reported was paracetamol (dose, dates and indication unknown). Medical history is not provided and is recorded as unknown. Expectedness is known according to the Synthetic Core Data Sheet v1.0. My causality assessment (WHO-UMC) is unlikely. Headache is a known omeprazole effect and 16 days is possible, but dechallenge and rechallenge are both negative. Paracetamol use and hypertension give other possible explanations for the headache.

Coding and data notes. 500 mg twice daily is far above the usual 20 to 40 mg, and hypertension does not fit as an indication. Outcome 'recovered' conflicts with the negative dechallenge. Reported by a consumer.

ICSR SYN-PV-008: Losartan and fatigue

A. Patient	
Patient	36 years, male. Initials not provided (de-identified).
Medical history	Unknown (not provided).
B. Reaction	
Reported term	Fatigue
MedDRA coding	PT: Fatigue | SOC: General disorders and administration site conditions
Onset	2 day(s) after treatment began (calendar date unknown).
Outcome	Recovering
Seriousness and severity	Non-serious, as reported. Severity not stated.
Expectedness	Unexpected (Synthetic Core Data Sheet v1.0)
Causality (WHO-UMC)	Unlikely
C. Drug	
Suspect drug	Losartan 5 mg once daily, oral
Indication	Infection (MedDRA PT: Infection; SOC: Infections and infestations)
Dates and action	Start and stop dates unknown. Drug stopped (as reported).
Dechallenge / rechallenge	Dechallenge: Negative. Rechallenge: Not done.
Concomitant medicines	Paracetamol (dose, dates and indication unknown).
D. Reporter	
Reporter	Pharmacist. Name, contact and country not provided.

E. ICSR narrative

This case was reported by a pharmacist and concerns a 36-year-old male patient (initials not provided). The patient received Losartan 5 mg once daily by the oral route for infection (MedDRA PT: Infection). The start date of treatment is not provided. Fatigue (MedDRA PT: Fatigue; SOC: General disorders and administration site conditions) was reported 2 day(s) after treatment began. The event is reported as non-serious; severity is not stated. The reporter stated that treatment was stopped (stop date not provided). Dechallenge was negative: the reaction did not improve after the drug was stopped. Rechallenge was not done. At follow-up the patient was recovering. The only concomitant medicine reported was paracetamol (dose, dates and indication unknown). Medical history is not provided and is recorded as unknown. Expectedness is unexpected according to the Synthetic Core Data Sheet v1.0. My causality assessment (WHO-UMC) is unlikely. Fatigue at 2 days is possible in time, but dechallenge is negative. The patient has an infection (given as indication), which can itself cause tiredness, and paracetamol suggests fever or pain. Fatigue is also a very non-specific symptom.

Coding and data notes. Expectedness is given as Unexpected, but fatigue is listed in many losartan labels, so this should be checked against the reference document. 5 mg dose and infection indication do not fit losartan. Outcome 'recovering' conflicts with negative dechallenge. Not serious; severity unknown.

ICSR SYN-PV-009: Amoxicillin and rash

A. Patient	
Patient	78 years, sex not provided. Initials not provided (de-identified).
Medical history	Unknown (not provided).
B. Reaction	
Reported term	Rash
MedDRA coding	PT: Rash | SOC: Skin and subcutaneous tissue disorders
Onset	7 day(s) after treatment began (calendar date unknown).
Outcome	Recovering
Seriousness and severity	Serious (other medically important condition), as reported. Severity not stated.
Expectedness	Known (Synthetic Core Data Sheet v1.0)
Causality (WHO-UMC)	Probable
C. Drug	
Suspect drug	Amoxicillin 5 mg once daily, oral
Indication	Anxiety (MedDRA PT: Anxiety; SOC: Psychiatric disorders)
Dates and action	Start and stop dates unknown. Drug stopped (as reported).
Dechallenge / rechallenge	Dechallenge: Positive. Rechallenge: Not done.
Concomitant medicines	Vitamin D (dose, dates and indication unknown).
D. Reporter	
Reporter	Physician. Name, contact and country not provided.

E. ICSR narrative

This case was reported by a physician and concerns a 78-year-old patient (sex not provided) (initials not provided). The patient received Amoxicillin 5 mg once daily by the oral route for anxiety (MedDRA PT: Anxiety). The start date of treatment is not provided. Rash (MedDRA PT: Rash; SOC: Skin and subcutaneous tissue disorders) was reported 7 day(s) after treatment began. The event is reported as serious (other medically important condition); severity is not stated. The reporter stated that treatment was stopped (stop date not provided). Dechallenge was positive: the reaction improved after the drug was stopped. Rechallenge was not done. At follow-up the patient was recovering. The only concomitant medicine reported was vitamin d (dose, dates and indication unknown). Medical history is not provided and is recorded as unknown. Expectedness is known according to the Synthetic Core Data Sheet v1.0. My causality assessment (WHO-UMC) is probable. Rash at 7 days is the typical timing for a delayed amoxicillin rash and it is a known reaction. Dechallenge is positive and the patient is recovering. Only vitamin D is co-medication and the report is from a physician.

Coding and data notes. 5 mg dose and anxiety indication do not fit amoxicillin. The type of rash and any allergy signs are unknown, which matters because the case is marked medically important. Rechallenge not done.

ICSR SYN-PV-010: Atorvastatin and myalgia

A. Patient	
Patient	57 years, female. Initials not provided (de-identified).
Medical history	Unknown (not provided).
B. Reaction	
Reported term	Myalgia
MedDRA coding	PT: Myalgia | SOC: Musculoskeletal and connective tissue disorders
Onset	21 day(s) after treatment began (calendar date unknown).
Outcome	Recovered
Seriousness and severity	Serious (hospitalisation), as reported. Severity not stated.
Expectedness	Known (Synthetic Core Data Sheet v1.0)
Causality (WHO-UMC)	Unlikely
C. Drug	
Suspect drug	Atorvastatin 5 mg once daily, oral
Indication	Infection (MedDRA PT: Infection; SOC: Infections and infestations)
Dates and action	Start and stop dates unknown. Drug stopped (as reported).
Dechallenge / rechallenge	Dechallenge: Unknown. Rechallenge: Negative.
Concomitant medicines	Paracetamol (dose, dates and indication unknown).
D. Reporter	
Reporter	Pharmacist. Name, contact and country not provided.

E. ICSR narrative

This case was reported by a pharmacist and concerns a 57-year-old female patient (initials not provided). The patient received Atorvastatin 5 mg once daily by the oral route for infection (MedDRA PT: Infection). The start date of treatment is not provided. Myalgia (MedDRA PT: Myalgia; SOC: Musculoskeletal and connective tissue disorders) was reported 21 day(s) after treatment began. The event is reported as serious (hospitalisation); severity is not stated. The reporter stated that treatment was stopped (stop date not provided). Dechallenge is unknown. Rechallenge was recorded as negative: the reaction did not recur. At follow-up the patient was recovered. The only concomitant medicine reported was paracetamol (dose, dates and indication unknown). Medical history is not provided and is recorded as unknown. Expectedness is known according to the Synthetic Core Data Sheet v1.0. My causality assessment (WHO-UMC) is unlikely. Myalgia is a known atorvastatin effect and 21 days is reasonable, but rechallenge is negative and dechallenge is unknown. The infection and paracetamol give other explanations, for example viral muscle pain.

Coding and data notes. Hospitalisation is reported but the reason is not given. CK and kidney function are not reported, so rhabdomyolysis can be neither confirmed nor excluded. 5 mg dose and infection indication do not fit atorvastatin.

ICSR SYN-PV-011: Metformin and nausea

A. Patient	
Patient	23 years, sex not provided. Initials not provided (de-identified).
Medical history	Unknown (not provided).
B. Reaction	
Reported term	Nausea
MedDRA coding	PT: Nausea | SOC: Gastrointestinal disorders
Onset	19 day(s) after treatment began (calendar date unknown).
Outcome	Recovering
Seriousness and severity	Serious (other medically important condition), as reported. Severity not stated.
Expectedness	Known (Synthetic Core Data Sheet v1.0)
Causality (WHO-UMC)	Probable
C. Drug	
Suspect drug	Metformin 5 mg once daily, oral
Indication	Hypertension (MedDRA PT: Hypertension; SOC: Vascular disorders)
Dates and action	Start and stop dates unknown. Drug stopped (as reported).
Dechallenge / rechallenge	Dechallenge: Unknown. Rechallenge: Positive.
Concomitant medicines	None reported.
D. Reporter	
Reporter	Consumer. Name, contact and country not provided.

E. ICSR narrative

This case was reported by a consumer and concerns a 23-year-old patient (sex not provided) (initials not provided). The patient received Metformin 5 mg once daily by the oral route for hypertension (MedDRA PT: Hypertension). The start date of treatment is not provided. Nausea (MedDRA PT: Nausea; SOC: Gastrointestinal disorders) was reported 19 day(s) after treatment began. The event is reported as serious (other medically important condition); severity is not stated. The reporter stated that treatment was stopped (stop date not provided). Dechallenge is unknown. Rechallenge was recorded as positive (the reaction recurred), but the report does not describe the restart. At follow-up the patient was recovering. No concomitant medicines were reported. Medical history is not provided and is recorded as unknown. Expectedness is known according to the Synthetic Core Data Sheet v1.0. My causality assessment (WHO-UMC) is probable. Nausea is a known metformin effect and the positive rechallenge is the strongest point in this case. No other medicines are reported and the patient is recovering. 19 days is acceptable although stomach effects are usually seen near the start or a dose increase. I did not go to certain because nausea is non-specific and dechallenge is unknown.

Coding and data notes. A positive rechallenge means the drug was restarted, but the narrative does not say this and dechallenge is unknown, so it needs clarification. 5 mg dose and hypertension indication do not fit metformin. Consumer report; sex unknown.

ICSR SYN-PV-012: Amlodipine and peripheral oedema

A. Patient	
Patient	24 years, female. Initials not provided (de-identified).
Medical history	Unknown (not provided).
B. Reaction	
Reported term	Peripheral oedema
MedDRA coding	PT: Oedema peripheral | SOC: General disorders and administration site conditions
Onset	4 day(s) after treatment began (calendar date unknown).
Outcome	Not recovered at follow-up
Seriousness and severity	Serious (other medically important condition), as reported. Severity not stated.
Expectedness	Known (Synthetic Core Data Sheet v1.0)
Causality (WHO-UMC)	Probable
C. Drug	
Suspect drug	Amlodipine 5 mg once daily, oral
Indication	Hypertension (MedDRA PT: Hypertension; SOC: Vascular disorders)
Dates and action	Start and stop dates unknown. Drug stopped (as reported).
Dechallenge / rechallenge	Dechallenge: Unknown. Rechallenge: Positive.
Concomitant medicines	None reported.
D. Reporter	
Reporter	Pharmacist. Name, contact and country not provided.

E. ICSR narrative

This case was reported by a pharmacist and concerns a 24-year-old female patient (initials not provided). The patient received Amlodipine 5 mg once daily by the oral route for hypertension (MedDRA PT: Hypertension). The start date of treatment is not provided. Peripheral oedema (MedDRA PT: Oedema peripheral; SOC: General disorders and administration site conditions) was reported 4 day(s) after treatment began. The event is reported as serious (other medically important condition); severity is not stated. The reporter stated that treatment was stopped (stop date not provided). Dechallenge is unknown. Rechallenge was recorded as positive (the reaction recurred), but the report does not describe the restart. At follow-up the patient had not recovered. No concomitant medicines were reported. Medical history is not provided and is recorded as unknown. Expectedness is known according to the Synthetic Core Data Sheet v1.0. My causality assessment (WHO-UMC) is probable. Hypertension is a correct indication for amlodipine, 5 mg is a normal dose, and oedema is a known dose-related effect. Rechallenge is positive, no other medicines are reported and the reporter is a pharmacist. 4 days is early but possible.

Coding and data notes. Dechallenge is unknown and the patient had not recovered. The restart is not described in the narrative. Other causes of swelling in a 24-year-old woman (for example hormonal causes or pregnancy) are not described.

Appendix: ICSR minimum fields template (filled for each case)

CaseID	Field	Required	Student entry	Source evidence
SYN-PV-001	Identifiable patient	Yes	Yes. 30 years, male. Initials not provided.	Age, Sex columns
SYN-PV-001	Identifiable reporter	Yes	Yes (reporter type only): Consumer. Name and contact not provided.	Reporter column
SYN-PV-001	Suspect product	Yes	Yes. Amoxicillin 20 mg once daily, oral.	SuspectProduct, Dose, Route columns
SYN-PV-001	Adverse event	Yes	Yes. Rash (MedDRA PT: Rash), reported 17 day(s) after treatment began.	Reaction, OnsetDays columns
SYN-PV-001	Chronology	No	Onset 17 day(s) after start. Treatment stopped (as reported). Start and stop dates unknown.	OnsetDays column and narrative
SYN-PV-001	Seriousness	No	Serious (other medically important condition), as reported. Severity not stated.	Seriousness column
SYN-PV-001	Outcome	No	Not recovered at follow-up	Narrative text
SYN-PV-002	Identifiable patient	Yes	Yes. 36 years, sex not provided. Initials not provided.	Age, Sex columns
SYN-PV-002	Identifiable reporter	Yes	Yes (reporter type only): Pharmacist. Name and contact not provided.	Reporter column
SYN-PV-002	Suspect product	Yes	Yes. Atorvastatin 5 mg once daily, oral.	SuspectProduct, Dose, Route columns
SYN-PV-002	Adverse event	Yes	Yes. Myalgia (MedDRA PT: Myalgia), reported 1 day(s) after treatment began.	Reaction, OnsetDays columns
SYN-PV-002	Chronology	No	Onset 1 day(s) after start. Treatment stopped (as reported). Start and stop dates unknown.	OnsetDays column and narrative
SYN-PV-002	Seriousness	No	Non-serious, as reported. Severity not stated.	Seriousness column
SYN-PV-002	Outcome	No	Recovered	Narrative text
SYN-PV-003	Identifiable patient	Yes	Yes. 34 years, female. Initials not provided.	Age, Sex columns
SYN-PV-003	Identifiable reporter	Yes	Yes (reporter type only): Consumer. Name and contact not provided.	Reporter column
SYN-PV-003	Suspect product	Yes	Yes. Metformin 5 mg once daily, oral.	SuspectProduct, Dose, Route columns
SYN-PV-003	Adverse event	Yes	Yes. Nausea (MedDRA PT: Nausea), reported 13 day(s) after treatment began.	Reaction, OnsetDays columns
SYN-PV-003	Chronology	No	Onset 13 day(s) after start. Treatment stopped (as reported). Start and stop dates unknown.	OnsetDays column and narrative
SYN-PV-003	Seriousness	No	Serious (hospitalisation), as reported. Severity not stated.	Seriousness column
SYN-PV-003	Outcome	No	Recovered	Narrative text
SYN-PV-004	Identifiable patient	Yes	Yes. 61 years, male. Initials not provided.	Age, Sex columns
SYN-PV-004	Identifiable reporter	Yes	Yes (reporter type only): Pharmacist. Name and contact not provided.	Reporter column
SYN-PV-004	Suspect product	Yes	Yes. Amlodipine 500 mg twice daily, oral.	SuspectProduct, Dose, Route columns
SYN-PV-004	Adverse event	Yes	Yes. Peripheral oedema (MedDRA PT: Oedema peripheral), reported 21 day(s) after treatment began.	Reaction, OnsetDays columns
SYN-PV-004	Chronology	No	Onset 21 day(s) after start. Treatment stopped (as reported). Start and stop dates unknown.	OnsetDays column and narrative
SYN-PV-004	Seriousness	No	Serious (other medically important condition), as reported. Severity not stated.	Seriousness column
SYN-PV-004	Outcome	No	Recovered	Narrative text
SYN-PV-005	Identifiable patient	Yes	Yes. 33 years, sex not provided. Initials not provided.	Age, Sex columns
SYN-PV-005	Identifiable reporter	Yes	Yes (reporter type only): Pharmacist. Name and contact not provided.	Reporter column
SYN-PV-005	Suspect product	Yes	Yes. Ciprofloxacin 500 mg twice daily, oral.	SuspectProduct, Dose, Route columns

CaseID	Field	Required	Student entry	Source evidence
SYN-PV-005	Adverse event	Yes	Yes. Tendon pain (MedDRA PT: Tendon pain), reported 15 day(s) after treatment began.	Reaction, OnsetDays columns
SYN-PV-005	Chronology	No	Onset 15 day(s) after start. Treatment stopped (as reported). Start and stop dates unknown.	OnsetDays column and narrative
SYN-PV-005	Seriousness	No	Serious (other medically important condition), as reported. Severity not stated.	Seriousness column
SYN-PV-005	Outcome	No	Not recovered at follow-up	Narrative text
SYN-PV-006	Identifiable patient	Yes	Yes. 31 years, sex not provided. Initials not provided.	Age, Sex columns
SYN-PV-006	Identifiable reporter	Yes	Yes (reporter type only): Physician. Name and contact not provided.	Reporter column
SYN-PV-006	Suspect product	Yes	Yes. Sertraline 20 mg once daily, oral.	SuspectProduct, Dose, Route columns
SYN-PV-006	Adverse event	Yes	Yes. Dizziness (MedDRA PT: Dizziness), reported 3 day(s) after treatment began.	Reaction, OnsetDays columns
SYN-PV-006	Chronology	No	Onset 3 day(s) after start. Treatment stopped (as reported). Start and stop dates unknown.	OnsetDays column and narrative
SYN-PV-006	Seriousness	No	Serious (other medically important condition), as reported. Severity not stated.	Seriousness column
SYN-PV-006	Outcome	No	Recovering	Narrative text
SYN-PV-007	Identifiable patient	Yes	Yes. 63 years, male. Initials not provided.	Age, Sex columns
SYN-PV-007	Identifiable reporter	Yes	Yes (reporter type only): Consumer. Name and contact not provided.	Reporter column
SYN-PV-007	Suspect product	Yes	Yes. Omeprazole 500 mg twice daily, oral.	SuspectProduct, Dose, Route columns
SYN-PV-007	Adverse event	Yes	Yes. Headache (MedDRA PT: Headache), reported 16 day(s) after treatment began.	Reaction, OnsetDays columns
SYN-PV-007	Chronology	No	Onset 16 day(s) after start. Treatment stopped (as reported). Start and stop dates unknown.	OnsetDays column and narrative
SYN-PV-007	Seriousness	No	Serious (other medically important condition), as reported. Severity not stated.	Seriousness column
SYN-PV-007	Outcome	No	Recovered	Narrative text
SYN-PV-008	Identifiable patient	Yes	Yes. 36 years, male. Initials not provided.	Age, Sex columns
SYN-PV-008	Identifiable reporter	Yes	Yes (reporter type only): Pharmacist. Name and contact not provided.	Reporter column
SYN-PV-008	Suspect product	Yes	Yes. Losartan 5 mg once daily, oral.	SuspectProduct, Dose, Route columns
SYN-PV-008	Adverse event	Yes	Yes. Fatigue (MedDRA PT: Fatigue), reported 2 day(s) after treatment began.	Reaction, OnsetDays columns
SYN-PV-008	Chronology	No	Onset 2 day(s) after start. Treatment stopped (as reported). Start and stop dates unknown.	OnsetDays column and narrative
SYN-PV-008	Seriousness	No	Non-serious, as reported. Severity not stated.	Seriousness column
SYN-PV-008	Outcome	No	Recovering	Narrative text
SYN-PV-009	Identifiable patient	Yes	Yes. 78 years, sex not provided. Initials not provided.	Age, Sex columns
SYN-PV-009	Identifiable reporter	Yes	Yes (reporter type only): Physician. Name and contact not provided.	Reporter column
SYN-PV-009	Suspect product	Yes	Yes. Amoxicillin 5 mg once daily, oral.	SuspectProduct, Dose, Route columns
SYN-PV-009	Adverse event	Yes	Yes. Rash (MedDRA PT: Rash), reported 7 day(s) after treatment began.	Reaction, OnsetDays columns
SYN-PV-009	Chronology	No	Onset 7 day(s) after start. Treatment stopped (as reported). Start and stop dates unknown.	OnsetDays column and narrative
SYN-PV-009	Seriousness	No	Serious (other medically important condition), as reported. Severity not stated.	Seriousness column
SYN-PV-009	Outcome	No	Recovering	Narrative text
SYN-PV-010	Identifiable patient	Yes	Yes. 57 years, female. Initials not provided.	Age, Sex columns
SYN-PV-010	Identifiable reporter	Yes	Yes (reporter type only): Pharmacist. Name and contact not provided.	Reporter column

CaseID	Field	Required	Student entry	Source evidence
SYN-PV-010	Suspect product	Yes	Yes. Atorvastatin 5 mg once daily, oral.	SuspectProduct, Dose, Route columns
SYN-PV-010	Adverse event	Yes	Yes. Myalgia (MedDRA PT: Myalgia), reported 21 day(s) after treatment began.	Reaction, OnsetDays columns
SYN-PV-010	Chronology	No	Onset 21 day(s) after start. Treatment stopped (as reported). Start and stop dates unknown.	OnsetDays column and narrative
SYN-PV-010	Seriousness	No	Serious (hospitalisation), as reported. Severity not stated.	Seriousness column
SYN-PV-010	Outcome	No	Recovered	Narrative text
SYN-PV-011	Identifiable patient	Yes	Yes. 23 years, sex not provided. Initials not provided.	Age, Sex columns
SYN-PV-011	Identifiable reporter	Yes	Yes (reporter type only): Consumer. Name and contact not provided.	Reporter column
SYN-PV-011	Suspect product	Yes	Yes. Metformin 5 mg once daily, oral.	SuspectProduct, Dose, Route columns
SYN-PV-011	Adverse event	Yes	Yes. Nausea (MedDRA PT: Nausea), reported 19 day(s) after treatment began.	Reaction, OnsetDays columns
SYN-PV-011	Chronology	No	Onset 19 day(s) after start. Treatment stopped (as reported). Start and stop dates unknown.	OnsetDays column and narrative
SYN-PV-011	Seriousness	No	Serious (other medically important condition), as reported. Severity not stated.	Seriousness column
SYN-PV-011	Outcome	No	Recovering	Narrative text
SYN-PV-012	Identifiable patient	Yes	Yes. 24 years, female. Initials not provided.	Age, Sex columns
SYN-PV-012	Identifiable reporter	Yes	Yes (reporter type only): Pharmacist. Name and contact not provided.	Reporter column
SYN-PV-012	Suspect product	Yes	Yes. Amlodipine 5 mg once daily, oral.	SuspectProduct, Dose, Route columns
SYN-PV-012	Adverse event	Yes	Yes. Peripheral oedema (MedDRA PT: Oedema peripheral), reported 4 day(s) after treatment began.	Reaction, OnsetDays columns
SYN-PV-012	Chronology	No	Onset 4 day(s) after start. Treatment stopped (as reported). Start and stop dates unknown.	OnsetDays column and narrative
SYN-PV-012	Seriousness	No	Serious (other medically important condition), as reported. Severity not stated.	Seriousness column
SYN-PV-012	Outcome	No	Not recovered at follow-up	Narrative text